Lutetium-177 oxodotreotide (Lutathera / peptide receptor radionuclide therapy (PRRT))

Specialist receptor-targeted radiation treatment This guide covers the Irish NCCP G1/G2 somatostatin receptor (SSTR)-positive gastroenteropancreatic neuroendocrine tumour (GEP-NET) regimen. A receptor scan, specialist eligibility review and individual radiation-safety plan are required.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

peptide receptor radionuclide therapy (PRRT) administration with kidney protection

Active medicines: Lutetium-177 oxodotreotide + Lysine / arginine amino acids

Emetogenic risk: Moderate

Medicine	Dose / route	When
5-HT3 antagonist antiemetic	Agent and dose prescribed by the nuclear-medicine service	At least 30 minutes before the amino-acid infusion National Cancer Control Programme (NCCP) regimen recommendation; additional antiemetics if needed under local policy.
Lysine / arginine amino-acid infusion	Formulation and volume prescribed by the specialist service	Start 30 minutes before Lutathera and continue for four hours Required kidney protection. Check potassium beforehand; fluid/renal/cardiac risk may require an individual plan.

Additional phase-specific support

- A lysine/arginine infusion is required for kidney protection and begins before Lutathera. Follow the service's fluid and toileting advice. Tell staff about heart or kidney problems; do not use a generic fluid target.
- PRRT has moderate emetogenic potential. A 5-HT3 anti-sickness medicine is given at least 30 minutes before the amino-acid infusion, with additional treatment if needed. Tell staff promptly if nausea starts.
- Long-acting somatostatin analogue (SSA) injections can interfere with receptor targeting. NCCP uses a 30-day interval before PRRT and short-acting injections are withheld for 24 hours. The referring neuroendocrine tumour (NET) consultant decides your exact stop and restart plan, balancing hormone control. Never stop injections on your own.
- The medical physicist gives individual written and verbal advice. Contact with young children or pregnant people may need restriction for up to three weeks, and close-contact work may require up to two weeks away. Arrange transport, sleeping, toileting, laundry and caring responsibilities before admission.
- Radiation safety: individual medical-physicist instructions and official radiation card; the SACTCheck wallet card does not replace them.
- The long treatment information describes earlier additional post-course blood tests and service scan timings; reconcile the written service plan with the NCCP minimum monitoring sequence.
- Do not restart SSA injections, prescribe steroids or add electrolyte/fluid treatment without the relevant specialist plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-11-05 · SACTCheck v0.84.37 · Regimen-level drafts; prescriber and oncology-pharmacy verification required